



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : لبنى محمد عبد العزيز محمد

العنوان : 59 ش الرشيد العجوزة

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 26211100104104 رقم التليفون : 01228478410

العمر (تاريخ الميلاد) : 10-11-1962 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: لبنى محمد عبد العزيز محمد National ID: 26211100104104	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

لبنى
محمد عبدالعزيز محمد
٥٩ رشيد
العجوزة - الجيزة

١٩٦٢/١/١٩ ٢٦٢١١٠٠١٠٤١٠٤
LA7406011

١١١٧٤٢٦٨٤٠
١٤٤٨٩٧٨٤١٠

٨/١٢ عيسى

٢٠٢٥/٠٣ ٢٦٢١١٠٠١٠٤١٠٤
ربة منزل
أنثى مسلمة أرملة
١ البطاقة سارية حتى ٢٠٣٢/٠٣/١٧

١

اسم المريض بالإنجليزية: Lobna Mohamed Abdalaziz Mohamed	كود المريض: 98359
تاريخ التقرير: 2026/08/12 10:20:40 AM	السن: ٦٣ سنة, ٩ شهر

10

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The Left breast shows an area of globular asymmetry occupies the upper outer quadrant with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By Ultrasound it corresponds to UOQ area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber , it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast: Left upper outer quadrant moderately suspicious lesion. (BIRADS 4 b) , for histopathology
- Right breast (BIRADS 1): normal study.

*Much obliged,
Dr. Hekmat Samir, MD*



قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: ابني محمد عبدالعزيز محمد Age: 64 years Sex: female
Request type: مبادره Sub request type:
Hospital no: Pathology no.:806-2026
Department: صحة مرآه Date of result:
Referred by: د/امل Date of biopsy:12/8/2026
Nature of specimen:tru-cut biopsy I.D. Number:26211100104104
Clinical diagnosis: Left breast lesion

Gross pathology :

Multiple grayish white tissue cores up to 0.5cm long, totally submitted.

Immunohistochemical Technique

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against:

ER- P63

Microscopic picture:

Section examined revealed breast core tissue showed extensive ductal carcinoma insitu formed of dilated ducts with inner proliferated ductal cells showed low to intermediate grade, comedo and solid subtypes. Also seen micro invasive component < 1mm, formed of clusters of anaplastic ductal cells with moderate anaplasia.

Immunohistochemical Result

ER: Strong nuclear staining in 90% of tumor cells (Allred Score 8/8).

P63: Positive in myoepithelial cells.

Diagnosis: Left Breast Lesion, Tru- Cut Biopsy:

Extensive Ductal Carcinoma Insitu with micro invasive Ductal Carcinoma component < 1mm.

Signature:

Dr.Mona Mohamed.M.D

Pathology Lab. Manager

BASMA REFAAT M.D

اسم المريض: الإنجليزية: Lobna Mohamed Abdalaziz Mohamed	كود المريض: 98359
تاريخ التقرير: 2026/08/12 10:20:40 AM	السن: ٤٣ سنة, ٩ شهر

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The Left breast shows an area of globular asymmetry occupies the upper breast with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By Ultrasound it corresponds to an area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) it extends from the region of 11 to 1 CLOCK , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber , it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Left breast:** Left upper breast moderately suspicious lesion. (BIRADS 4 b), for histopathology
- **Right breast (BIRADS 1):** normal study.

*Much obliged,
Dr. Hekmat Samir, MD*

اسم المريض بـ: إنجليزية: Lobna Mohamed Abdalaziz Mohamed	كود المريض: 98359
تاريخ التقرير: 2026/08/12 10:20:40 AM	السن: ٦٢ سنة, ٩ شهر

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The Left breast shows an area of globular asymmetry occupies the upper breast with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By **Ultrasound** it corresponds to an area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) it extends from the region of 11 to 1 CLOCK , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber , it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Left breast:** Left upper outer quadrant moderately suspicious lesion. (**BIRADS 4 b**), for histopathology
- **Right breast (BIRADS 1):** normal study.

*Much obliged,
Dr. Hekmat Samir, MD*

د. مصطفى
روسي
أخصائي أمراض النساء



مبادرة صحة المرأة المصرية
لدعم صحة المرأة المصرية

الهيئة العامة
لصحة
المرأة

09/23/2026

MDT FORM

Name:	ليلى محمد عبدالعزى محمد	MRN:	13672
Age:	64 Y	ID:	26211100104104
Menopausal Sta	post	Phone:	01228478410
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	DM , 4 cardiac stents last 2 yrs ago ,	Reason Of MDT:	new

Clinical Data:

BX: LT DCIS with micro invasion

ER: 8/8 **PR:** **HER2:** **KI67:**

Sonomammography:

ACR B
lt breast area 8x5 cm
11-1 o'clock
nonspecific axillary LNs
rt breast normal
Mets Workup:
CT :
pulmonary nodule 6mm for follow up
bilateral axillary + pericardial LNs ??
rt pubic bone small sclerotic lesion 6mm with no cortical disruption
no evidence of adrenal or liver mets

omnla essayy

History:

clinically:
lt breast central mass with bloody nipple discharge no palpable axillary Lns
for possibility of CT revision ??
then if free for LT SM + SLNB

omnla essayy

Recommendations:

MDT Outcome & Plan

Review CT CAP (Dr.Nermeen Ramadan) as pt has DCIS...if no evidence of metastases, proceed with Lt SM and SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/نبينا لاشين	د/لبنى عسوي				

د. مصطفى
روسي
أخصائي أمراض النساء
23/9/2026

about:blank

اسم المريض: <u>إحسانية</u> Mohamed	كود المريض: 98359
تاريخ التقرير: 2026/08/12 10:20:40 AM	السن: ٢٣ سنة ٩ شهر

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The left breast shows an area of globular asymmetry occupies the upper breast with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By Ultrasound it corresponds to an area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) it extends from the region of 11 to 1 CLOCK , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber , it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast: Left upper breast moderately suspicious lesion. (BIRADS 4 b), for histopathology
- Right breast (BIRADS 1): normal study.

*Much obliged,
Dr. Hekmat Samir, MD*

①



قسم الباثولوجيا

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: لبي محمد عبدالعزيز محمد

Age: 64 years

Sex: female

Request type: ملابرة

Sub request type:

Hospital no:

Pathology no.: 806-2026

Department: صدره مرأى

Date of result:

Referred by: د. لعل

Date of biopsy: 12/8/2026

Nature of specimen: tru-cut biopsy

I.D. Number: 26211100104104

Clinical diagnosis: Left breast lesion

Gross pathology:

Multiple grayish white tissue cores up to 0.5cm long, totally submitted.

Immunohistochemical Technique

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against:

ER- P63

Microscopic picture:

Section examined revealed breast core tissue showed extensive ductal carcinoma insitu formed of dilated ducts with inner proliferated ductal cells showed low to intermediate grade, comedo and solid subtypes. Also seen micro invasive component < 1mm, formed of clusters of anaplastic ductal cells with moderate anaplasia.

Immunohistochemical Result

ER: Strong nuclear staining in 90% of tumor cells (Allred Score 8/8).

P63: Positive in myoepithelial cells.

Diagnosis: Left Breast Lesion, Tru- Cut Biopsy:

Extensive Ductal Carcinoma Insitu with micro invasive Ductal Carcinoma component < 1mm.

Signature:

Dr.Mona Mohamed.M.D

Pathology Lab. Manager

ELASMA REFAAT M.D

Patient's Name:

Lubna Mohammed Abdul Aziz Mohammed

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

CLINICAL INDICATION: breast cancer workup.

COMPARISON: No previous study for comparison.

TECHNIQUE: Multiplanar CT with IV contrast CAP.

Non contrast and delayed phase.

CT CHEST

FINDINGS:

Bilateral axillary lymph nodes

largest in left side in level ii measures about 7 mm.

BONES: right pubic bone small sclerotic subchondral lesion with 900HU.

TRACHEA: Normal diameter and wall thickness, with no evidence of stenosis.

BRONCHI: Patent, with no evidence of bronchiectasis.

LUNGS: right upper lobe pulmonary GGO nodule 6 mm.

Another right lower lobe subpleural calcified nodule 6.5mm.

Bilateral few subpleural atelectatic bands.

No alveolar infiltration or interstitial thickening.

MEDIASTINUM: No mediastinal mass lesion or lymph node enlargement.

HILUM: No hilar mass lesion or lymph node enlargement.

CARDIAC CHAMBERS

No chamber dilatation.

Coronary arteries calcification.

PERICARDIUM: No pericardial thickening or effusion.

3 Pericardial lymph nodes the largest 9x8mm.

AORTA (ascending, arch and descending): Normal diameter, with no evidence of aortic aneurysm.

PULMONARY TRUNK AND MAIN PULMONARY ARTERIES: Normal diameter, with no evidence of pulmonary hypertension.

PLEURA: No pleural effusion.

ABDOMEN AND PELVIS

LIVER:

Size: average size

Surface: Smooth and regular.

Hepatic focal lesion: No solid or cystic mass lesion.

Portal vein: Patent.

Hepatic biliary system: Not dilated.

SPLEEN: average size. No splenic focal lesion.

KIDNEYS: Normal size, parenchymal thickness, density and enhancement. Normal contrast excretion on delayed images. No calculi or back pressure changes on either side. The perinephric fat planes are preserved.

ADRENAL GLANDS: both are average size, with no mass lesion on either side.

Pancreas: Normal size and density. No pancreatic mass lesion. The pancreatic duct is not dilated.

ABDOMINAL AORTA: Unremarkable. No aneurysmal outpouching or dissection.

Patient's Name:

Lubna Mohammed Abdul Aziz Mohammed

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

IVC: Unremarkable.

LYMPH NODES:

paraaortic lymph node 9.5x5.5mm.

No retroperitoneal, mesenteric or pelvic lymphadenopathy.

SMALL AND LARGE BOWEL: Unremarkable by CT criteria.

URINARY BLADDER: Normal wall thickness. No intravesical stone or mass.

RECTUM: Normal wall thickness. The perirectal fascia is preserved.

REPRODUCTIVE SYSTEM: unremarkable

PERTONEUM: No ascites.

IMPRESSION:

- Right pulmonary nodules as described for follow up.
- Bilateral axillary and Pericardial lymph nodes as described.
- right pubic bone small sclerotic subchondral lesion 6mm with no cortical disruption.
- No evidence of adrenal or liver metastasis.

DR Nada

(2)

Patient's Name:

Lubna Mohammed Abdul Aziz Mohammed

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

CLINICAL INDICATION: breast cancer workup.

COMPARISON: No previous study for comparison.

TECHNIQUE: Multiplanar CT with IV contrast CAP.

Non contrast and delayed phase.

CT CHEST

FINDINGS:

Bilateral axillary lymph nodes

largest in left side in level ii measures about 7 mm.

BONES: right pubic bone small sclerotic subchondral lesion with 900HU.

TRACHEA: Normal diameter and wall thickness, with no evidence of stenosis.

BRONCHI: Patent, with no evidence of bronchiectasis.

LUNGS: right upper lobe pulmonary GGO nodule 6 mm.

Another right lower lobe subpleural calcified nodule 6.5mm.

Bilateral few subpleural atelectatic bands.

No alveolar infiltration or interstitial thickening.

MEDIASTINUM: No mediastinal mass lesion or lymph node enlargement.

HILUM: No hilar mass lesion or lymph node enlargement.

CARDIAC CHAMBERS

No chamber dilatation.

Coronary arteries calcification.

PERICARDIUM: No pericardial thickening or effusion.

3 Pericardial lymph nodes the largest 9x8mm.

AORTA (ascending, arch and descending): Normal diameter, with no evidence of aortic aneurysm.

PULMONARY TRUNK AND MAIN PULMONARY ARTERIES: Normal diameter, with no evidence of pulmonary hypertension.

PLEURA: No pleural effusion.

ABDOMEN AND PELVIS

LIVER:

Size: average size

Surface: Smooth and regular.

Hepatic focal lesion: No solid or cystic mass lesion.

Portal vein: Patent.

Hepatic biliary system: Not dilated.

SPLEEN: average size. No splenic focal lesion.

KIDNEYS: Normal size, parenchymal thickness, density and enhancement. Normal contrast excretion on delayed images. No calculi or back pressure changes on either side. The perinephric fat planes are preserved.

ADRENAL GLANDS: both are average size, with no mass lesion on either side.

Pancreas: Normal size and density. No pancreatic mass lesion. The pancreatic duct is not dilated.

ABDOMINAL AORTA: Unremarkable. No aneurysmal outpouching or dissection.

Patient's Name:

Lubna Mohammed Abdul Aziz Mohammed

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

WC: Unremarkable.

LYMPH NODES:

paraaortic lymph node 9.5x5.5mm.

No retroperitoneal, mesenteric or pelvic lymphadenopathy.

SMALL AND LARGE BOWEL: Unremarkable by CT criteria.

URNARY BLADDER: Normal wall thickness. No intravesical stone or mass.

RECTUM: Normal wall thickness. The perirectal fascia is preserved.

REPRODUCTIVE SYSTEM: unremarkable

PERTONEUM: No ascites.

IMPRESSION:

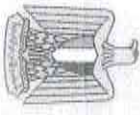
- Right pulmonary nodules as described for follow up.
- Bilateral axillary and Pericardial lymph nodes as described.
- right pubic bone small sclerotic subchondral lesion 6mm with no cortical disruption.
- No evidence of adrenal or liver metastasis.

DR Nada



1977

LA7406011



البطاقة مدارية حتى ٢٠٣٢/٠٣/١٧



2/2

اسم المريض بالإنجليزية: Lobna Mohamed Abdalaziz Mohamed	كود المريض: 98359
تاريخ الفحص: 2026/08/12 10:20:40 AM	السن: ٤٢ سنة ، ٩ شهر

110

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The Left breast shows an area of globular asymmetry occupies the upper outer quadrant with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By Ultrasound it corresponds to UOQ area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber , it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast: Left upper outer quadrant moderately suspicious lesion. (BIRADS 4 b) , for histopathology
- Right breast (BIRADS 1): normal study.

*Much obliged,
Dr. Hekmat Samir, MD*



N H T M I R I

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: ليلي محمد عبد العزيز محمد

Age: 64 years

Sex: female

Request type: مبداء

Sub request type:

Hospital no:

Pathology no.: 806-2026

Department: صحة بر ل

Date of result:

Referred by: د/ل

Date of biopsy: 12/8/2026

Nature of specimen: tru-cut biopsy

I.D. Number: 26211100104104

Clinical diagnosis: Left breast lesion

Gross pathology :

Multiple grayish white tissue cores up to 0.5cm long, totally submitted.

Immunohistochemical Technique

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against:

ER- P63

Microscopic picture:

Section examined revealed breast core tissue showed extensive ductal carcinoma insitu formed of dilated ducts with inner proliferated ductal cells showed low to intermediate grade, comedo and solid subtypes. Also seen micro invasive component < 1mm, formed of clusters of anaplastic ductal cells with moderate anaplasia.

Immunohistochemical Result

ER: Strong nuclear staining in 90% of tumor cells (Allred Score 8/8).

P63: Positive in myoepithelial cells.

Diagnosis: Left Breast Lesion, Tru-Cut Biopsy:

Extensive Ductal Carcinoma Insitu with micro invasive Ductal Carcinoma component < 1mm.

Signature:

Dr.Mona Mohamed.M.D

Pathology Lab. Manager

BASMA REFAAT M.D

اسم المريض: الدكتور: Lobna Mohamed Abdalaziz	كود المريض: 98359
اسم المريض: Mohamed	السن: ٢٣ سنة, ٩ شهر
تاريخ الفحص: 2026/08/12 10:20:40 AM	

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The Left breast shows an area of globular asymmetry occupies the upper breast with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By Ultrasound it corresponds to an area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) it extends from the region of 11 to 1 CLOCK , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber , it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast: Left upper breast moderately suspicious lesion. (BIRADS 4 b), for histopathology
- Right breast (BIRADS 1): normal study.

Much obliged,
Dr. Hekmat Samir, MD

اسم المريض : (خطير) : Lobna Mohamed Abdalaziz Mohamed	كود المريض : 98359
تاريخ الفحص : 2026/08/12 10:20:40 AM	السن : ٢٣ سنة , ٩ شهر

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 64 years old female with left breast lump.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- The Left breast shows an area of globular asymmetry occupies the upper breast with combined architectural distortion , this area measures approximately about 8 x 5 cm along the maximum dimensions
- By Ultrasound it corresponds to an area of non-mass heterogeneous parenchyma with dilated ducts (containing stagnant secretions) it extends from the region of 11 to 1 CLOCK , with the largest of these ducts located in the retroareolar region along with 12 : 00 clock measures about 5 mm in caliber ,it has a thickened irregular wall with soft tissue component within measures about 3 mm x 2 mm
- Normal nipple/areolar complex.
- Non-specific axillary LNs.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast: Left upper outer quadrant moderately suspicious lesion. (BIRADS 4 b), for histopathology
- Right breast (BIRADS 1): normal study.

*Much obliged,
Dr. Hekmat Samir, MD*

٢٠٤١٠٤ - ١١/١٢/٢٠٢١

لينا محمد عبد العزيز

Advanced Clinical Exam

الذي الأيسر

الذي الأيمن

طبيعي	كراخ أو تشقير في اتجاه الحمة
بروز الأوردة	تقرحات بالجاذ أو الحمة
تشقير في الشكل أو الحجم	افرازات غير طبيعية
حدوث تجوف	الشعور بالبرام بالأبط
طفح	الشعور بسمك عام
تجاعيد بالجاذ	ألم بالثدي
تحول الجاذ لما يقببه قشر البرتقال	قدم محسوس
الإحصار	تم إستكمال الثدي X

Maximum mass dimension (mm):

Side:

- o Right
- o left
- o Bilateral

Quadrant:

- o UOQ
- o UIQ
- o LOQ
- o LIQ

Central and retroareolar

Axilla

Imaging study requested:

- o Sonography
- o Mammosonography
- o MRI breasts

طبيعي	كراخ أو تشقير في اتجاه الحمة
بروز الأوردة	تقرحات بالجاذ أو الحمة
تشقير في الشكل أو الحجم	افرازات غير طبيعية
حدوث تجوف	الشعور بالبرام بالأبط
طفح	الشعور بسمك عام
تجاعيد بالجاذ	ألم بالثدي
تحول الجاذ لما يقببه قشر البرتقال	قدم محسوس
الإحصار	تم إستكمال الثدي X

غير خاضعة للتأمين

٩١٢
جنت